LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Mr SURYA KANT TIWARI**		
DOB/Age/Gender	: 70 Y/Male	Report STATUS	: Final Report
Patient ID / UHID	: 16003647/OF16003647	Barcode NO	: 58000785
Referred BY	: Dr. SELF WALK-IN	Sample Type	: Spot Urine
Sample Collected	: Mar 23, 2026, 08:53 AM	Report Date	: Mar 23, 2026, 01:08 PM.

NABL-M(EL)T-03100
MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Urine Routine and Microscopic Examination

Test Description	Value(s)	Unit(s)	Reference Range
Physical Examination *			
Volume * *visual*	30	ml	-
Colour * *visual*	Pale yellow	-	Pale yellow
Transparency * *visual*	Clear	-	Clear
Deposit * *visual*	Absent	-	Absent
Chemical Examination *			
Reaction (pH) *Double Indicator*	6.0	-	4.5 - 8.0
Specific Gravity *Ion Exchange*	**1.005 L***	-	1.010 - 1.030
Urine Glucose (sugar) *Oxidase / Peroxidase*	Negative	-	Negative
Urine Protein (Albumin) *Acid / Base Colour Excahnge*	Negative	-	Negative
Urine Ketones (Acetone) *Legals Test*	Negative	-	Negative
Blood *Peroxidase Hemoglobin*	Negative	-	Negative
Leucocyte esterase *Enzymatic Reaction*	Negative	-	Negative
Bilirubin Urine *Coupling Reaction*	Negative	-	Negative
Nitrite *Griless Test*	**Positive H***	-	Negative
Urobilinogen *Ehrlichs Test*	Normal	-	Normal
Microscopic Examination *			
Pus Cells (WBCs) *	2-4	/hpf	0 - 5
Epithelial Cells *	1-2	/hpf	0 - 4
Red blood Cells *	Absent	/hpf	Absent
Crystals *	Absent	-	Absent
Cast *	Absent	-	Absent
Yeast Cells *	Absent	-	Absent
Amorphous deposits *	Absent	-	Absent
Bacteria *	**Present H***	-	Absent
Protozoa *	Absent	-	Absent
Comment *	RESULT RECHECKED,		

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)
(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Ravi Rani Mishra
MD Pathology
Consultant Pathologist

Booking Centre :- Walk-In Redcliffe Labs- Prayagraj, Redcliffe Lifetech Pvt Ltd, 33/29 Lowther Road, George Town, Prayagraj, UP 211002
Processing Lab :- Redcliffe Lifetech Pvt Ltd, 33/29 Lowther Road, George Town, Prayagraj, UP - 211002

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Test Description	Value(s)	Unit(s)	Reference Range
	KINDLY CORRELATE CLINICALLY.		

Interpretation:

URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.

Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.

Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.

Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.

Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.

Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.

pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.

Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.

Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.

Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.

*** End Of Report ***

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)

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2. It is to be presumed that the tests performed pertain to the specimen/sample attributed to the Customer's name or identification. It is presumed that the verification particulars have been cleared out by the customer or his/her representation at the point of generation of said specimen / sample. It is hereby clarified that the reports furnished are restricted solely to the given specimen only.

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